

**I:** So now we'll start with the actual interesting questions.

**I:** So the first one is about familiarity with AI.

**I:** So how familiar are you with AI technologies in general?

**P:** I think that also depends on how you define familiarity, but I mean, I know about chat GPT and I know how to look things up.

**P:** I've used it a little bit, but mostly for fun.

**P:** Like, I don't know, with friends.

**P:** Someone is having a baby and we ask chat GPT, these are the names of the parents and their other kids.

**P:** You know, what do you recommend?

**P:** Like name, you know, stuff like that.

**P:** Kind of writing vows or whatever.

**P:** So more like for fun in that way.

**P:** Sometimes...

**P:** I've used it to research something, but I don't use it a lot.

**P:** I know a lot of people who use it a lot more than I do.

**P:** I think I'm also a little bit hesitant to use it because I know it takes quite a lot of energy.

**P:** I know it often leads to mistakes.

**P:** And I sometimes come across it when

**P:** I also teach sometimes for the [POST-ACADEMIC PROGRAM].

**P:** And it has happened that I've gotten a report from a student and I'm like, this is not you, this is AI.

**P:** And I hate that because often I ask them to write reflections about their thinking and if I get something that's made by AI, then it's no use.

**P:** So sometimes it annoys me a little bit.

**I:** yeah understandable um so uh in your organization have you also introduced ai at all or have you not

**P:** no no not really, so i work by myself but there's there's like two sort of companies that i work with and one of them i work with because if you have your own practice it's pretty hard to

**P:** to give people mental health care that's paid for by insurance because there's like so many things that you need to do before mental health care is like yeah sure we'll pay for the sessions that you do so there's this company that I work with and like through them

**P:** I take all of the boxes.

**P:** Yeah, I can, you know, I can do session that are paid for by mental health care.

**P:** And I know that they have like a pilot going on.

**P:** Where I think AI sort of, you know, listens to your sessions and then makes a summary.

**P:** And I have not applied for that.

**I:** Okay, Yeah

**P:** And that's come across.

**I:** Yeah.

**I:** So what company is this?

**I:** That's like overarching?

**P:** Yeah, it's called [COMPANY] and it's a company that specifically works with psychologists that work by themselves and that have experience and that are able to work

**P:** with people like in the first line of the basic mental health care the sessions that are paid for by the government but that by themselves are they're not recognized by mental health care as professionals and they do the whole administrative side of things

**P:** Yeah.

**P:** So it's, it's, they also saw like, there's so many waiting lists, so many people waiting for treatment.

**P:** And there's also a big group of psychologists that are able to treat these people, but they're not allowed because there's, you know, some boxes they don't tick.

**P:** So they kind of, you know, they, they build an organization to kind of connect these two, which I think is, is really good.

**P:** Yeah.

**I:** Okay.

**I:** So to finish this question off, can you give us a,

**I:** score of 1 to 10 on how familiar you are with AI?

**P:** I'm guessing that's a two or three.

**P:** Okay.

**I:** Okay.

**I:** Then for the next question, um, this is about the diagnostics and assessment use.

**I:** Um, so what are your thoughts on AI being used in diagnostic or assessment processes?

**P:** um yeah how would you use them

**I:** I mean they're they're like um i also don't want to say too much because i don't want to steer the conversation right but um let's let's say in in psychology you want to diagnose people right and um you have like tests for that or you have like an intake right and

**I:** there might be some opportunities for AI to help in that so for example you mentioned that AI could summarize things so that could be a thing that is helpful but there might also be other things it could do right like it could also say hey this person has this disorder, there could be a wide range of applications.

**I:** So what are your thoughts and maybe some things that you think oh that might be something that it shouldn't be doing or it should be doing this or it could help in this or not in this and

**P:** yeah yeah okay yeah well one thing you said is like that we want to diagnose people well i don't necessarily want to diagnose people sometimes it can be useful but um we

**P:** need to do it because the insurance companies want us to put a label on it so they can decide to pay or not.

**P:** That's mostly why we do the diagnosis.

**P:** And using AI for that, I think there's

**P:** part of it that you could do, or you could maybe use it to sort of form your opinion a little bit further.

**P:** But I can also imagine like forming a diagnosis is, I mean, you're, of course, you're looking at symptoms that somebody is experiencing, but there's a lot of symptoms that can be like a symptom of so many different diagnoses.

**P:** So if, for example, I would work with an athlete that's like really strict about

**P:** sleeping times and eating habits and, you know, what you should do and shouldn't do.

**P:** And it's like really rigorous about that.

**P:** Then there might also be a chance that AI says, oh, yeah, this is autism.

**P:** Since, you know, there's some symptoms in there.

**P:** But I think those are also things that you sometimes need in elite sports to perform well.

**P:** So in the sports context, it's not necessarily a symptom of autism.

**P:** While in another context, it could be.

**P:** I'm not sure whether AI can truly understand these different contexts that people can be in.

**P:** Because it feels like it mostly looks at the symptoms and where can I find these symptoms together in an overview.

**P:** And then we'll probably put a diagnosis on it like that.

**P:** And then...

**P:** So I think you could maybe use it to get some ideas.

**P:** So you could maybe, I don't know, put a list of the things that you're thinking or the symptoms in AI and see what it comes up with like diagnosis wise.

**P:** That might be a little bit quicker as well than getting your DSM and looking at all the different diagnoses yourself.

**P:** So maybe it can save a little bit of time in that regard.

**P:** Um, but there's, I'm not sure what to do with these.

**P:** Sometimes there's also these things that you sense or that you see, you see the body language, you see nonverbal stuff and you can describe some of that of course, but there's also some of these things that are hard to put into words, but you do register them as a psychologist, which I think it's hard to, yeah, to

**P:** make AI understand the relevance of that yeah so I think in that regard it might be hard so to make an actual diagnosis I wouldn't trust AI to do that but to give you some ideas sort of brainstorm with you as like a person I might do that yeah

**I:** so for diagnosing do you actually use the DSM like for every case or uh

**P:** do you mean the book itself or yeah yeah yeah not i mean there's some diagnoses that i've i've you know used so much that i don't i mean that you understand yeah it's pretty clear like yeah some anxiety disorders or depressive disorders there you know i've

**P:** I've used them quite a lot.

**P:** So I know the most important symptoms and sometimes it's just really clear.

**P:** Also like post-traumatic stress disorder, for example, I see that's quite a lot.

**P:** And then, um, yeah, that's not too, you know, certainly not too hard to, to use, but, um, sometimes when it's something that, um, it's when it's, when it's, it's symptoms that are a bit more, I don't know, either

**P:** abnormal or i don't come across them too often or there's a combination where i'm like i'm not sure what is going on yeah and then i might use the and i might use the dsm or use the internet actually i think i use the internet more than i use the book itself but then i try to look for reliable websites that have the same criteria on the rest of the symptoms itself

**I:** So is there a specific websites you, you look at, or is it just Googling like, oh, these are some symptoms or like, how does this go?

**P:** I could do both.

**P:** There's like a Dutch website, which, you know, like the guidelines in the, in the mental health care.

**P:** And, um, you can use that pretty well.

**P:** There's all types of different disorders on there and it's all according to the DSM V.

**P:** So I try to use that.

**P:** I mean, I know you can Google, but if you Google, you also get so much rubbish.

**P:** Yeah, there's a lot of things that you see there or you get like websites where people are trying to diagnose themselves, you know, so sometimes that's not really useful.

**P:** I might use it and then see what comes out and see which websites are reliable, usable, and I might choose those.

**P:** um like nowadays you get like this little ai prompt at the top yeah um i use that sometimes but for work for putting diagnoses for using diagnoses then i'm pretty careful using that yeah yeah in my spare time i might use that more with a little prompt but but it's less important stuff but yeah for making diagnosis yeah

**I:** Okay, so then for the next question is about monitoring and progress tracking.

**I:** So how do you view the role of AI in tracking patient progress over time?

**P:** I don't think it has a role in my work.

**P:** I think the only way to do that is to

**P:** let AI listen to all of your sessions and then ask AI whether they see, I don't know, a difference or like make a graph or you can probably ask AI to do stuff like that.

**P:** But I don't trust it enough to let it listen to my sessions.

**P:** And I think that if I would ask AI to make a summary of my sessions, then it would not be

**P:** A summary that I would find useful was that it would probably just be a summary of what we talked about and the things that I write down as a summary is it's more interpretations.

**P:** It's more points of view.

**P:** It's a little bit of a summary on how the client's doing, but also if I do like an exercise with a client.

**P:** then I don't need a whole description of the exercise I did because I know how I did the exercise.

**P:** What I would write down is how a client responded, what would be the key moments or the most emotional moments or what would be the takeaway message for a client.

**P:** I would write those things down.

**P:** I think it's hard to teach AI to sort of get the things from the session that I need.

**P:** So I wouldn't really see how I would

**P:** how I could use like a summary from AI.

**P:** Maybe you should write like a really specific prompt on how to do that.

**P:** But I sort of write along a little bit when I'm doing my sessions and it also helps me to stay focused.

**P:** So it also has, it has a purpose that I'm doing that during the session as well.

**P:** So yeah.

**P:** Yeah.

**I:** Yeah.

**I:** And so you mentioned that you don't trust AI in doing these kinds of things.

**I:** Like, is there a specific,

**I:** part of the trust that you think is like a limiting factor?

**I:** Like what type of trust is this?

**P:** Well, I promise my clients confidentiality and if there's something listening along and I'm not completely sure where that data goes or if there's someone who can buy that or

**P:** then I can't promise that.

**I:** Yeah, so it's mostly focused on the privacy part, right?

**I:** Yeah.

**I:** Okay, I forgot.

**I:** Let's see if I have a follow up for this.

**I:** Oh, yeah.

**I:** So do you actually do any monitoring with your clients as well, or is that not really used that much?

**P:** You mean on the progress that someone...

**P:** Verbally, I do that a lot, of course.

**P:** If you look at questionnaires or things like that, often there's one that I do when we start and when we finish.

**P:** Because often, of course, you hope that at the end, the client verbally tells you that he or she is doing better.

**P:** But it's also really helpful to have a questionnaire that

**P:** that solidifies that, that shows you that the person is actually doing better.

**P:** Because often people at the end, they've sort of forgotten how they were feeling at the beginning.

**P:** So sometimes when you look at those two questionnaires at the end, the beginning and what's happening now, often they're like, oh yeah, that's the way I felt.

**P:** Even though sometimes it's like four months in the past, they don't have a very clear recollection of that.

**P:** So I think that's a helpful tool um but i don't use like questionnaire every session or yeah

**I:** yeah and also not a uh not a questionnaire in the middle because with the previous uh psychologists i've talked to they do like a start and end in the middle um but you mainly focus on start and end right?

**P:** well for this questionnaire i do but the questionnaire that i'm talking about now is a questionnaire that's

**P:** that measures symptoms that people experience.

**P:** So that's, you know, that's another very positive way to look at it, of course, because we're looking at, you know, how many symptoms of a disorder are there, or how many depressive symptoms or anxiety symptoms, or, you know, look at that.

**P:** Verbally, what I often do is also somewhere halfway through, we sort of have an evaluation where we're looking at how you think everything's going.

**P:** What do you think of the sessions?

**P:** What do you think of the method that we're using?

**P:** How far do you think you are in your process?

**P:** I sometimes ask them to sort of give me sort of a percentage on where they are at.

**P:** So I do that verbally sometimes.

**P:** but not at a very sort of set moment so often that's also either something that occurs sort of naturally in um in the treatment um or i'm curious at a certain point or or at a point where like or not completely sure what the next step is and then i do an evaluation as well so yeah

**I:** uh okay so then for the next question it's about administrative and practical support um so how do you feel about ai generating drafts or reports or assisting with scheduling and reminders

**P:** drafts and reports um, I think you could use it for that.

**P:** Then you also need to look at the confidentiality issues of course.

**P:** Maybe if you would do that without any names or whatever, then you could ask AI to do that.



**P:** But then, yeah, I think I could look into doing that.

**P:** But I'm also thinking now is that often if I have to write like a report for someone, then often there's already like a format to do that.

**P:** I've done that more often.

**P:** So there's certain sentences that I sort of use again, but in a different context.

**P:** So it doesn't take me that much time and I don't need to write that many reports that I'm like, I need a solution for this.

**P:** Um, so.

**P:** Yeah, so I'm not really looking for that, but I think maybe you could use it for reports if you really look at anonymity well and make sure that the confidentiality is taken care of.

**P:** Then I'm guessing you could use it for reports.

**P:** Yeah, then I would also always read it over.

**I:** Yeah, yeah, yeah.

**P:** And then, yeah.

**P:** I'm curious if I would be, if I would be happy with it, but then I would, I would just have to try that to figure out whether it works or not.

**I:** Yeah.

**I:** Okay.

**I:** And so, um, also with, uh, with the mind on like scheduling, so you, you work with this company, right?

**I:** I don't know if they plan your schedule or if you do it yourself or.

**P:** Yeah.

**P:** Yeah.

**P:** Nobody touches my schedule.

**P:** No, I like to do that myself.

**P:** I know there's also some colleagues that, and I think that I have a program that also is able to do that, but there's some colleagues who have a system where clients can see their calendar and they can put an appointment in there themselves.

**P:** That idea sounds horrible to me.

**P:** I don't know.

**P:** I like to...

**P:** have that control over my own calendar.

**P:** And what I often do with clients is to plan ahead at the end of the session.

**P:** And then often we put, often I have two appointments in the future.

**P:** I'm guessing if, for example, at the moment that you're making the appointments, if you could, I don't know, put on AI and AI could put appointments in the calendar straight away the moment that you

**P:** plan them with the clients and make sure that they receive a confirmation email or something like that.

**P:** Well, that, that would save me 30 seconds, I guess.

**P:** So, I mean, that would be possible.

**P:** Um, but, um, yeah, so I'm guessing in that way you could, you could use it.

**P:** Um, but what I do now, we just plan at the end of the session and then put them in the calendar straight away.

**P:** And I have,

**P:** I have a program that if I put them in, clients automatically receive a confirmation email on the day before they receive a reminder.

**P:** Yeah.

**P:** So that is also something that happens automatically.

**P:** So I don't need to do that myself.

**P:** So I think that works pretty well.

**P:** But I'm guessing in that way, I mean, I'm open to ideas.

**P:** If I could do this part in an easier way, then sure.

**I:** Yeah.

**I:** And you mentioned that the idea of your clients being able to plan their own appointments would be horrible.

**I:** So that's describing the existing system that your colleagues or people you know already are using, right?

**P:** Yeah.

**P:** Yeah, some of them are using it.

**P:** There's a lot of colleagues that don't use that because they don't like it.

**P:** Because it's always nicer to sort of have the control or often when, I don't know, if this Friday, for example, I would only have appointments in the morning and somebody would want to make another appointment, then I would first give them the choice of having like the first appointment of the afternoon instead of the last one.

**P:** Because otherwise I would have like,

**P:** A hole in my calendar of two or three hours.

**P:** So then I try to do it that way so I have a little bit more control over my calendar and over the rest of my day.

**P:** So that's why I like to keep that control.

**I:** Okay, now for the next question, it's about psychoeducation and between session tools.

**I:** So what are your thoughts on using AI for psycho education or therapeutic exercises between sessions?

**P:** I think you could do that.

**P:** I mean, well, there's a lot of things online that you could use already.

**P:** So there's e-learning, for example, that clients can do.

**P:** I, well, quite often I give clients exercises to do in the meantime that I explain to them and they can do it themselves.

**P:** I'm guessing if sometimes there's clients who have a hard time, you know, remembering to do something or, um, so, well, what we sometimes do in the session is that they put like reminders in their phone during the session.

**P:** So they won't forget to do the exercise.

**P:** Maybe if you could use

**P:** AI for that to give them reminders or something that could be useful.

**P:** But yeah, I think there's already programs on the market that also help with that.

**P:** So I'm not really sure why I would necessarily need AI for that.

**P:** But I'm guessing you could use it for that.

**I:** So what type of homework do you give them?

**I:** Is this like a e-learning environment or is it also just normal, like physical world homework?

**P:** Yeah.

**P:** Yeah.

**P:** It's more physical things.

**P:** Yeah.

**P:** No, it's, it's more, um, it depends a little bit on what we're doing in the session.

**P:** So what I quite regularly do with clients is often they,

**P:** For example, when we've had one session or two sessions, often there's something happening.

**P:** So maybe they have anxiety in a certain situation or they have a fear of failure or whatever.

**P:** And often the question at the beginning is, why is the client experiencing that?

**P:** So we want to figure out which circumstances in their lives have sort of

**P:** caused this is a big word but are part of you know this is occurring so what I often then do is sort of look back into their lives chronologically and see what has happened over the years and then sometimes it happens that there are certain years they don't really remember or I ask them like you know do you know what you were like as a kid or at school you know did you

**P:** Did you always try to do really well or ask some questions about that?

**P:** Some people know that really well, but there's also people who are like, yeah, I'm not really sure.

**P:** So then sometimes an assignment can be to have a conversation with one of their parents or somebody else who knew them in that time and get some information on what they thought they were like.

**P:** So there can be exercises that people get.

**P:** But exercises can also be more like behavioral therapy type exercises where people start practicing things that they find scary.

**P:** And then we often start with like really small exercises and then slowly we make them a little bit bigger.

**P:** So sort of like exposure exercises.

**P:** Sometimes you can do that together in a session as well, but sometimes they can just start

**P:** doing that by themselves and then just start off really small.

**P:** And I work with ACT a lot, acceptance and commitment therapy.

**P:** And for that, I also have a lot of exercises that people do.

**P:** So, for example, making a list of all of the rules they have for themselves.

**P:** So a lot of people have a lot of rules like how they should live and what they should and shouldn't do and that they

**P:** Should always be tough and always be strong and never cry in public and always be nice, you know, rules like that.

**P:** So sometimes I give them an exercise to try to note down as many rules as they can think of.

**P:** So, yeah, sometimes it's also exercises like that.

**P:** Sometimes it's really doing stuff, sometimes writing stuff down, practicing things, practicing, focusing on their feelings or mindfulness exercises.

**I:** Yeah.

**I:** And what would you think about if you might be using in the future AI for psychoeducation or between sessions what would you think about it when it contradicts you as a therapist

**P:** I think that's probably very easy to contradict a psychologist because there's so many different ways to do things.

**P:** So different types of treatments were, yeah.

**P:** So I think it's, if you,

**P:** If you look at things from the behavioral therapy side or from the acceptance and commitment therapy side, there's some things in both of these therapies that sort of clash a little bit.

**P:** But they're both methods that work well.

**P:** And I actually think that from all of the methods that you could use in psychology, almost all of them sort of have the same success rate.

**P:** of the most used ones, and it's mostly the therapists themselves that fix the difference and they strengthen the bond, the relationship they have with the client

**P:** It's actually what determines a successful outcome.

**P:** So I think it's very easy for AI to contradict me, but then I would be curious on which grounds and from which point of view they would contradict me.

**P:** If it's just, this is wrong, you should do that, then you can't really do anything with that.

**P:** So if there's a good explanation on why you could look at it in a different way, then sure, I'd be open to that.

**P:** I'm guessing that you also do that when you have intervision with colleagues.

**P:** Sometimes they also see things in a different way, and that's really helpful to gain a different perspective.

**P:** So if it could give me a different perspective with a solid foundation, then sure, that would be good.

**P:** If it would just give me

**P:** another perspective without any foundation, I would probably be like, yeah, fine.

**I:** Yeah.

**I:** Um, okay.

**I:** So let's move on to the next question.

**I:** It's about, uh, risks and limitations.

**I:** Um, so you already mentioned a few, like the data privacy stuff and just now with the kind of like a transparency, like if you, if you get an answer from AI and there has to be some founding

**I:** idea behind it right it cannot just say this is the answer I don't know where I got it from right so are there any other risks or limitations that you think could come up with using AI

**P:** I think it's mostly like the the interpretations of things it's

**P:** I'm not sure if AI has the capacity to take into account all of these different contexts and these different types of people.

**P:** Because I think in psychology, you learn a lot about how things work for a lot of people.

**P:** But then if you're working and you do individual work, it's always with an individual who

**P:** sometimes has the same experience as that group, but often it's also slightly different because it's, I mean, all of those things are, you know, they're sort of the products of maybe tens of thousands of people and then like it's their collective experience, which is always different from one individual experience.

**P:** And that makes it really hard because I think AI mostly uses sort of more collective general things.

**I:** Yeah.

**P:** And it's just really hard to use that for one individual and really take their perspective into account.

**P:** So yeah, like interpretations and sensitivity part of sessions, I think that's really hard for AI to do because it mostly works with information and that you should, you have to make

**P:** sort of verbal or into written texts.

**P:** And yeah, there's also quite a lot of information that's just not, you cannot really put that into written texts too well.

**P:** Especially if there's, for example, sometimes in sessions, there's something happening between you and a client where like the client has patterns and they respond in a certain way, but you as a psychologist, you also have your own patterns and sometimes you also react to that.

**P:** And then you get like a very interesting,

**P:** thing happening that you can talk about with the client.

**P:** But I don't think AI has the capacity to understand that because then you have to understand the whole history of the psychologist and also of the client.

**P:** I think that's too many individual experiences.

**P:** I think that's too hard at the moment at least for AI.

**I:** Okay, so then for the next question, it's about client perspective and therapeutic alliance.

**I:** So how do you think clients respond to AI being used in their treatment?

**P:** I'm not really sure.

**P:** I haven't talked about it a lot with clients.

**P:** No.

**P:** Then I would also have to ask, for example, the company that I work with, you know, how is their pilot doing, cause they are doing like a pilot?

**I:** Mm-hmm.

**P:** And I'm guessing these psychologists have to ask their clients, is it okay if AI listens to our session?

**P:** And I haven't heard any results about the pilot yet, so I'm not sure how people respond.

**P:** So I'm not sure.

**P:** All I can say about this is sort of things that I would think people would think, which is not really a good answer because it's really subjective.

**P:** So...

**P:** Yeah, I think if I talk to people about, as I've talked to some people about, you know, using AI for summaries of sessions, and then I think I've straightaway given my perspective, like, well, at the moment, I would not do that.

**P:** I would not, like the confidentiality part.

**P:** And I think most people agree with me, but I'm not sure whether they would also agree completely if they wouldn't have heard my opinion first.

**P:** So, yeah, I don't know.

**P:** I don't know how clients would respond to that.

**I:** I do want a score on this.

**I:** If we have a 1 to 10 score of how likely you think clients would want to use or would be willing to use AI in their sessions.

**P:** Yeah.

**P:** I think that really the

**P:** think for a lot of clients that would really depend on how I explain it yeah so I think if people trust their psychologist and if you have a very clear objective in using it and for why it's helpful for clients then I think a lot of clients would be willing to use it so then I think

**P:** if you, yeah, then I think maybe 60 or 70% would probably say, oh, well in that case, then it's, then it's fine.



**P:** And if you can take away concerns, then it might be fine.

**P:** Yeah.

**P:** So I think that really depends.

**P:** Yeah.

**P:** Yeah.

**P:** So I think you can, you can steer that pretty well as a psychologist.

**I:** Yeah.

**I:** I think it definitely would be important to, uh, like,

**I:** Yea so it's important to explain why you would be wanting to use AI and what benefits it would bring.

**P:** Yea

**I:** um okay so for the last question it's about professional identity autonomy and future conditions we kind of mentioned this a bit so which aspects of your work should remain strictly human and what would you need to feel confident about using ai safely in practice i think there's a lot of things you already mentioned with um that you just don't think that

**I:** AI can capture certain contexts, right?

**I:** Or the one-on-one relationship.

**I:** So anything you want to expand on?

**P:** Yeah.

**P:** I mean, I think they're experimenting also with having like an AI person doing the whole session.

**P:** I think that would be, I think that would be hard.

**P:** That would be, I think, yeah, pretty.

**P:** Quickly, that might feel like a little bit fake or a little bit, I mean, I think what most clients need is to feel heard by someone and to feel like somebody is really there for them and takes the time for them.

**P:** And yeah, I think it would be harder to achieve that with an AI person.

**P:** So in sessions themselves, I think it's important to do that with a person

**P:** And I think, for example, doing it online doesn't really change that a lot because then it's still another person who sits in front of you and sometimes says something weird or makes a mistake or you can laugh together about it.

**P:** Or I think AI would also maybe be pretty to the point or, yeah, I don't know.

**P:** I think conversations with a human are not always to the point or sometimes you have like a tangent and that's okay.

**P:** Now you go back to the conversation and that helps, I think, to keep it natural.

**P:** So I think that part should definitely remain human.

**P:** And what was the second half of your question?

**I:** And what would you need to feel confident about using AI safely in practice?

**P:** Yeah, I think you would need to understand like 100% where does this data go?

**P:** Is it stored somewhere?

**P:** Is there a way to hack this?

**P:** Is there...

**P:** I think AI is also really intangible for a lot of people.

**P:** They have no idea what happens to all of this stuff and all of this information.

**P:** And where, I mean, I think most people know that the information that AI has, it also comes from the internet.

**P:** It comes from different websites that it goes to, but there's so much rubbish on the internet.

**P:** So how can you be sure that the foundation is

**P:** is good.

**P:** You would also need to know where does it get its information from.

**P:** Yeah.

**P:** So, you probably need a lot more understanding.

**P:** Yeah.

**P:** And be sure that the data is completely secure, but that's hard in this world.

**I:** For sure.

**I:** Yeah.

**I:** I just had some ideas, but I forgot.

**I:** no i won't i won't come up with it um do you have anything else to add to this

**P:** no

**I:** no okay then i will stop the recording